

M2T6: Food Sensitivities and Elimination — 11 min

Video File M2T6-Sensitivities.mp4 · **Resources** Elimination Guide.pdf (sensitivity quiz, food-symptom diary, safe elimination and reintroduction plan) · **Xp** 90 total · Take Sensitivity Quiz 40 · Remove 1 Suspect Food 30 · Report Results in Forum 20

Target runtime 11 min · 1,384 spoken words · ≈ 9.2 min at 150 wpm · ≈ 10.3 min at a warm 135 wpm

0:00–0:30 **HOOK**

[VISUAL: Cynthia on camera.]

A woman I worked with had been bloated after dinner for so long she thought it was just what dinner felt like. She'd tried cutting bread on her own, felt a little better, and then stopped, because it was hard and she wasn't sure it was even doing anything. When she finally asked her doctor for one specific blood test, she got an answer that explained her symptoms, and that answer changed her care. Today I'm going to tell you which test that was, and why the order you do things in matters.

0:30–2:00 **WHY SENSITIVITIES MATTER FOR CONCEPTION**

[VISUAL: Slide: "Sensitivity · Intolerance · Allergy · Autoimmune: not the same thing."]

Let's be precise, because the internet isn't. A food allergy is an immune reaction that can be immediate and dangerous; that's the peanut-and-EpiPen kind. An intolerance means your body

lacks something it needs to digest a food, like lactose intolerance, where you're short on the enzyme that breaks down milk sugar. A sensitivity is a vaguer word for foods that reliably make you feel unwell, bloated, foggy, or sore, without a clear mechanism. And celiac disease is an autoimmune condition where gluten triggers the immune system to damage the lining of the small intestine.

Why does this matter before conception? Because inflammation and poor absorption reach the reproductive system. Celiac disease is the clearest example. A meta-analysis in Human Reproduction Update by Tersigni and colleagues found that undiagnosed celiac disease was more common among women with unexplained infertility and recurrent pregnancy loss, and that a strict gluten-free diet, once diagnosed, appeared to improve outcomes. Untreated celiac also impairs absorption of iron, folate, and other nutrients that a pregnancy depends on. So finding out isn't a lifestyle preference. It's part of preparing.

I also want to be honest about testing, because a lot of money changes hands in this area. Blood tests that measure IgG antibodies to dozens of foods are widely sold as food

sensitivity panels. The American Academy of Allergy, Asthma and Immunology, along with its European and Canadian counterparts, has stated that IgG results do not identify food sensitivity; those antibodies mostly reflect foods you've eaten, not foods that harm you. So a panel that tells you to avoid thirty foods is usually telling you what you had for lunch last month. The tools that actually work are less glamorous: a diary, a structured elimination, and a doctor who can order the right test for the right condition.

2:00–6:00 COMMON SENSITIVITIES: GLUTEN, DAIRY, AND THE REST

[VISUAL: Slide: "Gluten: three different conditions."]

Gluten first. There's celiac disease, which affects roughly one in a hundred people and is diagnosed by blood tests and, often, a biopsy. There's wheat allergy, which is an immune reaction to wheat proteins. And there's non-celiac gluten sensitivity, where people feel better without gluten but test negative for the other two. Here is the single most important thing in this training, and I want you to write it down: get tested for celiac disease before you remove gluten. The standard blood test looks for antibodies your body only makes while you're eating gluten. If you've been gluten-free for

weeks, the test can come back falsely normal, and you'll have lost the chance to learn something that matters for your health and your baby's. The National Institutes of Health and the Celiac Disease Foundation both stress this. So the order is: symptoms, doctor, test, then diet.

[VISUAL: Slide: "Dairy: intolerance vs allergy, and the fertility surprise."]

Dairy is more nuanced than most people think. Lactose intolerance is common, especially in adults of East Asian, African, and Indigenous ancestry, and it's uncomfortable but not dangerous; lactose-free milk, hard cheeses, and fermented dairy like kefir and yogurt are often well tolerated because much of the lactose is already broken down. Milk protein allergy is different and rarer in adults. And here's the surprise: in the Nurses' Health Study, published in Human Reproduction, women who ate full-fat dairy had a lower risk of anovulatory infertility, while low-fat dairy went the other direction. So I don't tell anyone to cut dairy for fertility. If dairy makes you unwell, that's a reason. Fertility, by itself, is not.

[VISUAL: Slide: "Other common suspects: eggs, soy, corn, FODMAPs, added sugar, alcohol, caffeine."]

Other common suspects: eggs, soy, corn, certain high-fermentation carbohydrates called FODMAPs that bother people with irritable bowel syndrome, and, honestly, the three that cause the most symptoms in the people I work with: added sugar, alcohol, and too much caffeine. Before you suspect a whole food group, it's worth asking whether the real culprit is the soda and the second glass of wine.

And it's worth asking about sleep and stress, too. Many of the symptoms people blame on food, bloating, fatigue after meals, brain fog, are also classic signs of poor sleep and a stressed nervous system, because both slow digestion and change the gut lining. If your symptom diary shows that your worst days follow your shortest nights, the food may be innocent. That's not a reason to ignore it. It's a reason to read the whole diary, not just the food column.

6:00–9:30 DETECTION AND THE ELIMINATION PROCESS

[VISUAL: Slide: Elimination Guide preview: sensitivity quiz, food-symptom diary, reintroduction schedule.]

So how do you find out what's bothering you? Three steps. Step one is the sensitivity quiz in the Elimination Guide: a symptom inventory covering digestion, energy, skin, mood, and cycles. It doesn't diagnose anything, but it shows you

where to look. Step two is a food-and-symptom diary for one week: what you ate, when, and how you felt two hours later and the next morning. Patterns show up in a diary that are invisible in memory. Step three, if a pattern points at a specific food, is a short, structured elimination.

Here's how to do that safely. Remove one suspect food, just one, completely, for two to three weeks. Not five foods; you'll never know which one mattered, and you'll be hungry and miserable. Keep everything else the same and keep the diary. If gluten is your suspect, this is where you stop and talk to your doctor first about celiac testing; if dairy is your suspect, talk to your doctor if you have any history of allergy.

Then reintroduce. Eat a normal portion of the food, once, and watch for three days, because some reactions are delayed. If nothing happens, it's probably not your problem, and you get to enjoy it again. If your symptoms come back, you have your answer, and it's an answer built on evidence rather than a headline.

Two guardrails. First, do not cut out whole food groups long-term without a registered dietitian or your doctor in the loop,

especially before conception, when you need every nutrient you can get; long-term unsupervised elimination is how people end up low in iron, calcium, and B vitamins. Second, if you have any history of disordered eating, please do this with a professional or skip the elimination entirely and focus on adding. I say that with love and from experience. Restriction can wake up old patterns, and nothing in this course is worth that.

Here's what a good conversation with your doctor sounds like, so you can walk in prepared. "I've been keeping a food and symptom diary for a week; here it is. I'm trying to conceive, so I want to rule out anything that affects absorption or inflammation. Could we test for celiac disease before I change my diet? Are there other tests you'd suggest, like iron, vitamin D, or thyroid? And if I do a short elimination of one food, is there anything I should watch for?" Most doctors welcome a patient who arrives with a diary and specific questions. The Elimination Guide has this conversation printed on its own page.

9:30–10:30 CYNTHIA'S ADVICE AND HEART-LED CLOSE

[VISUAL: Cynthia on camera.]

My advice, in one line: listen to your body, but get it a translator. Your symptoms are real, and they're worth investigating; that's the listening part. The translator is a diary and a doctor, so that what you learn is true. The woman I told you about at the start spent a year guessing. One test, one diary, one conversation, and she stopped guessing.

This is for your future family. A body that absorbs its nutrients well, and isn't fighting a low-grade war with its dinner, is a body that has more to give to the life you're preparing for.

10:30–11:00 ACTION HANDOFF

[VISUAL: End card: "Elimination Guide PDF → Take the quiz (40 XP) → Remove 1 suspect food for 2–3 weeks (30 XP) → Report results in the forum (20 XP)."]

Download the Elimination Guide and take the sensitivity quiz for forty XP. If it points somewhere, remove one suspect food, with your doctor's input where I've flagged it, for thirty. Report what you found in the forum for twenty, so others can learn from your experiment. Next, Training Seven: supplements, and what actually has evidence behind it.

ACTION CALLOUT

RESOURCE Elimination Guide.pdf (sensitivity quiz, 7-day food-symptom diary, single-food elimination and 3-day reintroduction plan, doctor conversation checklist). **ACTION 1:** Take the sensitivity quiz (40 XP). **ACTION 2:** Remove one suspect food for 2–3 weeks, with medical input where flagged (30 XP). **ACTION 3:** Report your results in the forum (20 XP). **NEXT STEP:** Training 7 · Supplements for Fertility.